

100 Most Important Diseases & Tumors

High-Yield Buzzwords for Benign & Malignant Lesions

1. Glioblastoma Multiforme (GBM)

- Pseudopalisading necrosis on histology
- Butterfly glioma (crosses corpus callosum)
- GFAP positive (astrocytic origin)
- Most common primary malignant adult brain tumor
- Poor prognosis, rapid progression

2. Meningioma

- Psammoma bodies (calcifications)
- Whorled pattern of spindle cells
- Arachnoid cap cell origin, dural tail on MRI
- Most common benign adult brain tumor
- Often asymptomatic or presents with seizures

3. Schwannoma

- Antoni A (dense) and Antoni B (loose) areas
- Verocay bodies
- S-100 positive
- Often involves CN VIII (Cerebellopontine angle)
- Bilateral in Neurofibromatosis Type 2

4. Oligodendroglioma

- Fried egg appearance (cells with clear halos)
- Chicken-wire capillary pattern
- Often calcified (frontal lobe)
- Slow-growing tumor
- 1p/19q co-deletion indicates better response to therapy

5. Medulloblastoma

- Homer-Wright rosettes
- Drop metastases to spinal cord
- Most common malignant brain tumor in children
- Arises in cerebellar vermis
- Highly radiosensitive

6. Ependymoma

- Perivascular pseudorosettes
- Arises in 4th ventricle in children
- Can cause non-communicating hydrocephalus
- Rod-shaped blepharoplasts found near nucleus
- Poor prognosis

7. Hemangioblastoma

- Associated with Von Hippel-Lindau (VHL) syndrome
- Produces Erythropoietin (EPO) -> Polycythemia
- Foamy cells and high vascularity
- Cerebellum most common site
- Can present with ataxia

8. Craniopharyngioma

- Derived from Rathke's pouch remnants
- Motor oil-like fluid inside cysts
- Calcifications visible on imaging
- Causes bitemporal hemianopsia (optic chiasm compression)
- Most common childhood supratentorial tumor

9. Pilocytic Astrocytoma

- Rosenthal fibers (eosinophilic corkscrew fibers)
- GFAP positive
- Cystic lesion with a mural nodule
- Most common benign brain tumor in children (cerebellum)
- Excellent prognosis

10. Neuroblastoma

- Homer-Wright rosettes
- N-myc gene amplification (poor prognosis)
- Crosses the midline (unlike Wilms tumor)
- Elevated HVA/VMA in urine
- Bombesin and NSE positive

11. Wilms Tumor (Nephroblastoma)

- WT1 or WT2 gene mutation (Chr 11)
- Contains embryonic glomerular structures (triphasic)
- Large, palpable unilateral flank mass
- Does NOT cross the midline
- Associated with WAGR and Beckwith-Wiedemann syndromes

12. Renal Cell Carcinoma (Clear Cell)

- Polygonal cells with clear cytoplasm (lipids/ glycogen)
- VHL gene mutation (Chromosome 3)
- Painless hematuria, palpable mass, flank pain
- Paraneoplastic syndromes: EPO, PTHrP, ACTH
- Invades renal vein and IVC

13. Urothelial (Transitional Cell) Carcinoma

- Painless hematuria
- Risk factors: Smoking, Aniline dyes, Cyclophosphamide
- Fibrovascular cores on histology
- Field defect (high rate of recurrence)
- Most common bladder cancer

14. Squamous Cell Carcinoma of Bladder

- Associated with Schistosoma haematobium infection (Middle East)
- Chronic cystitis / prolonged catheterization risk
- Painless hematuria
- Keratin pearls on histology
- Presents late, poor prognosis

15. Prostate Carcinoma

- Arises in the peripheral zone (palpable on DRE)
- Elevated PSA (Prostate-Specific Antigen)
- Glandular crowding with prominent nucleoli
- Osteoblastic (bone-forming) metastases to spine
- Graded using Gleason scoring system

16. Seminoma

- Fried egg appearance (clear cytoplasm, central nuclei)
- Elevated Placental ALP (hCG may be slightly elevated)
- Most common testicular tumor
- Highly sensitive to radiotherapy
- Painless testicular enlargement

17. Yolk Sac Tumor (Endodermal Sinus)

- Schiller-Duval bodies (resemble primitive glomeruli)
- Highly elevated AFP (Alpha-fetoprotein)
- Most common testicular tumor in boys <3 years old
- Yellow, mucinous cut surface
- Can also occur in the ovary

18. Choriocarcinoma

- Markedly elevated hCG
- Cytotrophoblasts and syncytiotrophoblasts (no chorionic villi)
- Early hematogenous spread (lungs, brain)
- Can cause hyperthyroidism (hCG mimics TSH)
- Highly aggressive, but responds to chemo if gestational

19. Leydig Cell Tumor

- Reinke crystals (eosinophilic cytoplasmic inclusions)
- Produces androgens or estrogens
- Causes precocious puberty in boys
- Causes gynecomastia in men
- Golden-brown color on gross exam

20. Teratoma

- Contains elements from all 3 germ layers (ecto, meso, endo)
- Benign in females (mature cystic teratoma)
- Malignant in post-pubertal males
- Can contain teeth, hair, thyroid tissue (struma ovarii)
- AFP/hCG may be elevated if mixed

21. Fibroadenoma

- Most common benign breast tumor <35 years old
- Mobile, well-circumscribed 'breast mouse'
- Estrogen sensitive (grows during pregnancy)
- Composed of fibrous stroma and glandular tissue
- No increased risk of cancer

22. Phyllodes Tumor

- Leaf-like architecture with stromal hypercellularity
- Occurs in older, postmenopausal women
- Large, bulky breast mass
- Can be benign, borderline, or malignant
- Requires wide local excision

23. Intraductal Papilloma

- Most common cause of bloody nipple discharge
- Benign tumor within lactiferous ducts
- Fibrovascular core lined by epithelial cells
- Located beneath the areola
- Slight increased risk for cancer if multiple

24. Ductal Carcinoma In Situ (DCIS)

- Fills ductal lumen but NO basement membrane invasion
- Microcalcifications on mammography
- Comedonecrosis subtype has high grade cells with central necrosis
- Precursor to invasive ductal carcinoma
- Usually does not produce a palpable mass

25. Invasive Ductal Carcinoma

- Most common type of breast cancer
- Rock-hard, immobile mass
- Scirrhus, stellate morphology
- Nests and cords of cells (no Indian file pattern)
- Often presents with skin dimpling or nipple retraction

26. Invasive Lobular Carcinoma

- Indian file pattern (single-file rows of cells)
- Loss of E-cadherin expression
- No duct formation
- Frequently bilateral and multicentric
- Often lacks a distinct palpable mass

27. Paget Disease of the Breast

- Eczematous patches on nipple and areola
- Paget cells: large cells with clear halo in epidermis
- Almost always indicates underlying DCIS or invasive carcinoma
- PAS positive, Keratin positive
- Must differentiate from melanoma

28. Squamous Cell Carcinoma of Lung

- Centrally located (hilar mass)
- Keratin pearls and intercellular bridges
- Strongly associated with smoking
- Produces PTHrP -> Hypercalcemia
- Cavitating lesion on CXR

29. Small Cell Carcinoma of Lung

- Centrally located, highly aggressive
- Kulchitsky cells (small, dark blue cells)
- Neuroendocrine origin (Chromogranin A, NSE positive)
- Paraneoplastic syndromes: ACTH (Cushing), SIADH, Lambert-Eaton
- Treat with chemo, not surgery

30. Adenocarcinoma of Lung

- Most common lung cancer in non-smokers and overall
- Peripherally located
- Glandular pattern, mucin-producing
- Associated with EGFR, ALK, KRAS mutations
- Clubbing and hypertrophic osteoarthropathy

31. Carcinoid Tumor of Lung

- Well-differentiated neuroendocrine cells in nests
- Chromogranin A positive
- Excellent prognosis, rarely metastasizes
- Can cause carcinoid syndrome (flushing, diarrhea)
- Polypoid mass in bronchus

32. Mesothelioma

- Malignancy of the pleura
- Strongly associated with asbestos exposure
- Psammoma bodies seen on histology
- Calretinin positive
- Pleural thickening with bloody effusions

33. Hepatocellular Carcinoma (HCC)

- Elevated AFP (Alpha-fetoprotein)
- Risk factors: Hep B/C, Aflatoxin from Aspergillus, Cirrhosis
- Tends to invade hepatic and portal veins
- Can cause Budd-Chiari syndrome
- Hematogenous spread is common

34. Hepatic Adenoma

- Benign liver tumor
- Associated with Oral Contraceptive Pill (OCP) use
- High risk of rupture and intraperitoneal hemorrhage during pregnancy
- Subcapsular location
- Usually regresses with cessation of OCPs

35. Hepatic Hemangioma

- Most common benign liver tumor
- Cavernous vascular spaces lined by endothelium
- Do NOT biopsy (risk of fatal hemorrhage)
- Usually asymptomatic and incidental finding
- Peripheral nodular enhancement on contrast CT

36. Cholangiocarcinoma

- Adenocarcinoma of the bile ducts
- Associated with Primary Sclerosing Cholangitis (PSC)
- Associated with Clonorchis sinensis (liver fluke)
- Presents with painless jaundice
- Poor prognosis

37. Esophageal Squamous Cell Carcinoma

- Upper 2/3 of the esophagus
- Risk factors: Alcohol, Smoking, Hot liquids, Achalasia
- Progressive dysphagia (solids to liquids)
- Weight loss and hoarseness
- Keratin pearls on biopsy

38. Esophageal Adenocarcinoma

- Lower 1/3 of the esophagus
- Arises from Barrett esophagus (intestinal metaplasia)
- Risk factors: GERD, Obesity
- Progressive dysphagia
- Most common esophageal cancer in the West

39. Gastric Adenocarcinoma (Intestinal Type)

- Associated with Helicobacter pylori infection
- Dietary risks: Nitrosamines, smoked foods
- Common on lesser curvature of stomach
- Presents with weight loss, early satiety, and anemia
- Forms glandular structures

40. Gastric Adenocarcinoma (Diffuse Type)

- Signet ring cells (mucin pushes nucleus to periphery)
- Linitis plastica (leather bottle stomach) due to desmoplasia
- E-cadherin mutation
- Not associated with H. pylori
- Metastasizes to bilateral ovaries (Krukenberg tumor)

41. Gastrointestinal Stromal Tumor (GIST)

- Arises from Interstitial cells of Cajal (pacemaker cells)
- c-KIT (CD117) positive
- Spindle cell morphology
- Most common in the stomach
- Treated with Imatinib (tyrosine kinase inhibitor)

42. Familial Adenomatous Polyposis (FAP)

- APC gene mutation (Chromosome 5q)
- Autosomal dominant inheritance
- Thousands of colonic polyps by teenage years
- 100% risk of colorectal cancer
- Prophylactic colectomy is required

43. Colorectal Carcinoma

- Right-sided: Bleeding, iron deficiency anemia, weight loss
- Left-sided: Obstruction, 'apple-core' lesion on barium enema
- CEA used for monitoring recurrence
- Arises via APC/beta-catenin or microsatellite instability pathways
- Most common GI malignancy

44. Carcinoid Tumor (GI tract)

- Most commonly in the appendix or terminal ileum
- Salt and pepper chromatin
- Secrete serotonin (5-HT)
- Carcinoid syndrome only occurs if there are liver metastases
- Elevated 5-HIAA in urine

45. Pancreatic Adenocarcinoma

- Tumor marker CA 19-9
- Head of pancreas: Painless jaundice, Courvoisier sign
- Body/Tail: Weight loss, back pain
- Trousseau syndrome (migratory thrombophlebitis)
- Highly aggressive, poor survival rate

46. Papillary Thyroid Carcinoma

- Most common thyroid cancer
- Orphan Annie eye nuclei (empty appearing)
- Psammoma bodies
- Nuclear grooves
- Excellent prognosis, lymphatic spread

47. Follicular Thyroid Carcinoma

- Invades through tumor capsule or blood vessels
- Hematogenous spread (bones, lungs)
- Cannot be diagnosed by FNA (requires observing capsule)
- Uniform follicles
- Good prognosis if caught early

48. Medullary Thyroid Carcinoma

- Arises from Parafollicular C cells
- Secretes Calcitonin (can cause hypocalcemia)
- Amyloid stroma (Apple-green birefringence on Congo red)
- Associated with MEN 2A and 2B (RET mutation)
- Spindle-shaped cells

49. Anaplastic Thyroid Carcinoma

- Occurs in elderly patients
- Highly aggressive, rapid enlargement
- Pleomorphic giant cells and spindle cells
- Compresses trachea (dyspnea) and esophagus (dysphagia)
- Very poor prognosis

50. Pheochromocytoma

- Most common tumor of the adrenal medulla in adults
- Produces catecholamines
- Episodic hypertension, headaches, sweating, palpitations
- Elevated urine metanephrines and VMA
- 10% rule (bilateral, malignant, extra-adrenal)

51. Parathyroid Adenoma

- Most common cause of primary hyperparathyroidism
- Hypercalcemia and Hypophosphatemia
- 'Stones, bones, groans, and psychiatric overtones'
- Osteitis fibrosa cystica (brown tumors in bone)
- Treated with surgical excision

52. Pituitary Adenoma (Prolactinoma)

- Most common functioning pituitary tumor
- Bitemporal hemianopsia (optic chiasm compression)
- Females: Galactorrhea, amenorrhea
- Males: Decreased libido, impotence
- Treated medically with Cabergoline or Bromocriptine

53. Osteosarcoma

- Codman triangle and Sunburst appearance on X-ray
- Arises in metaphysis of long bones (distal femur/proximal tibia)
- Produces osteoid matrix
- Associated with Retinoblastoma and Li-Fraumeni syndrome
- Bimodal age distribution (teens and >65)

54. Ewing Sarcoma

- Onion skin periosteal reaction on X-ray
- Arises in diaphysis of long bones and pelvis
- t(11;22) translocation
- Homer-Wright rosettes, small blue cells
- Aggressive, common in boys <15 years old

55. Osteoid Osteoma

- Benign bone tumor (<2cm radiolucent nidus)
- Bone pain that is worse at night
- Pain is characteristically RELIEVED by NSAIDs/ Aspirin
- Common in cortex of long bones
- Osteoblastoma is similar but >2cm and not relieved by NSAIDs

56. Giant Cell Tumor of Bone

- Arises in the epiphysis of long bones (often knee)
- Soap bubble appearance on X-ray
- Multinucleated osteoclast-like giant cells
- Locally aggressive benign tumor
- Occurs in young adults (20-40 years)

57. Chondrosarcoma

- Malignant cartilaginous tumor
- Occurs in medulla of pelvis and central skeleton
- Common in older adults (>40 years)
- Expansile glistening mass in medullary cavity
- Does not respond well to chemotherapy

58. Osteochondroma

- Most common benign bone tumor
- Cartilage-capped bony exostosis
- Arises from metaphysis
- Grows away from the joint
- Rarely transforms to chondrosarcoma

59. Basal Cell Carcinoma (BCC)

- Most common skin cancer
- Pearly papule with telangiectasias
- Rolled borders and central ulceration
- Palisading nuclei at the periphery on histology
- Locally invasive but rarely metastasizes

60. Squamous Cell Carcinoma of Skin

- Second most common skin cancer
- Keratin pearls on histology
- Actinic keratosis is a precursor lesion
- Common on lower lip, hands, and face
- Risk factors: UV light, Immunosuppression, Arsenic

61. Melanoma

- S-100, HMB-45, and Melan-A positive
- ABCDE criteria (Asymmetry, Border, Color, Diameter, Evolving)
- BRAF V600E mutation common
- Breslow depth is the most important prognostic factor
- Highest risk of metastasis among skin cancers

62. Seborrheic Keratosis

- Benign squamous proliferation
- Stuck-on, greasy, warty appearance
- Horn cysts (keratin-filled) on histology
- Leser-Trélat sign (sudden onset of multiple SKs = GI malignancy)
- Very common in the elderly

63. Kaposi Sarcoma

- Associated with HHV-8 infection
- Common in HIV/AIDS patients
- Endothelial cell proliferation
- Spindle cells with slit-like vascular spaces
- Purple macules/plaques on skin and GI tract

64. Hodgkin Lymphoma (Nodular Sclerosing)

- Most common subtype of Hodgkin lymphoma
- Reed-Sternberg cells (Owl eyes appearance)
- CD15+ and CD30+
- Lacunar cells and collagen banding
- Presents with painless cervical lymphadenopathy

65. Burkitt Lymphoma

- t(8;14) translocation of c-myc gene
- Starry sky appearance on histology (macrophages tingible bodies)
- Endemic form: Jaw mass (EBV associated)
- Sporadic form: Abdominal mass
- Highly aggressive B-cell lymphoma

66. Follicular Lymphoma

- t(14;18) translocation of BCL-2 gene (inhibits apoptosis)
- Painless, waxing and waning lymphadenopathy
- Nodular proliferation of centrocytes and centroblasts
- Indolent course, difficult to cure
- Can transform to DLBCL

67. Diffuse Large B-Cell Lymphoma (DLBCL)

- Most common Non-Hodgkin Lymphoma in adults
- Aggressive, rapidly enlarging mass
- CD19+, CD20+ B-cells
- Can arise de novo or from Richter transformation of CLL
- Treat with R-CHOP regimen

68. Mantle Cell Lymphoma

- t(11;14) translocation of Cyclin D1 gene
- Promotes G1 to S phase cell cycle transition
- CD5+ and CD20+ B-cells
- Occurs in older males
- Very aggressive, poor prognosis

69. Multiple Myeloma

- CRAB: HyperCalcemia, Renal involvement, Anemia, Bone lytic lesions
- M spike on SPEP (usually IgG or IgA)
- Bence Jones proteinuria (free light chains)
- Rouleaux formation of RBCs on smear
- Plasma cells >10% in bone marrow

70. Acute Lymphoblastic Leukemia (ALL)

- Most common childhood cancer
- TdT positive (marker of pre-T and pre-B cells)
- PAS positive
- Associated with Down syndrome (>5 years old)
- T-ALL presents as a mediastinal mass in teenagers

71. Acute Myeloid Leukemia (AML)

- Auer rods (peroxidase positive cytoplasmic inclusions)
- MPO positive
- t(15;17) in APML subtype (treat with ATRA - Vitamin A)
- High risk of DIC (especially APML)
- Adults (median age 65)

72. Chronic Myelogenous Leukemia (CML)

- t(9;22) Philadelphia chromosome
- BCR-ABL fusion gene (tyrosine kinase overactivity)
- Massive splenomegaly and increased basophils
- Low LAP (Leukocyte Alkaline Phosphatase) score
- Treat with Imatinib

73. Chronic Lymphocytic Leukemia (CLL)

- Most common leukemia in adults
- Smudge cells on peripheral smear
- CD5+ B-cells
- Often asymptomatic (incidental lymphocytosis)
- Can cause autoimmune hemolytic anemia

74. Leiomyoma (Fibroid)

- Most common benign gynecologic tumor
- Whorled pattern of smooth muscle bundles
- Estrogen-sensitive (grows in pregnancy, shrinks in menopause)
- Presents with menorrhagia and pelvic pressure
- Multiple discrete tumors common

75. Leiomyosarcoma

- Malignant smooth muscle tumor of uterus
- Arises DE NOVO (not from fibroids)
- Postmenopausal bleeding
- Necrosis, nuclear atypia, and high mitotic rate
- Single large bulky mass

76. Endometrial Carcinoma

- Most common gynecologic malignancy in the US
- Presents with postmenopausal bleeding
- Risk factors: Unopposed estrogen, Nulliparity, Obesity, PCOS
- PTEN mutation associated with endometrioid type
- Preceded by endometrial hyperplasia

77. Cervical Carcinoma

- Driven by HPV 16 and 18 (E6 inhibits p53, E7 inhibits Rb)
- Squamous cell carcinoma is most common
- Presents with post-coital bleeding
- Can cause renal failure (lateral invasion blocks ureters)
- Preventable via Pap smear screening and HPV vaccine

78. Serous Cystadenocarcinoma

- Most common malignant ovarian tumor
- Bilateral frequently
- Psammoma bodies on histology
- BRCA-1 and BRCA-2 mutations increase risk
- Complex cysts with thick septations and solid components

79. Mucinous Cystadenocarcinoma

- Malignant ovarian tumor producing mucin
- Lined by atypical, multilayered mucin-secreting epithelium
- Risk of pseudomyxoma peritonei (jelly belly)
- Large multiloculated cystic masses
- Poor prognosis if ruptured

80. Dysgerminoma

- Most common malignant germ cell tumor of ovary
- Fried egg appearance on histology
- Elevated LDH and hCG
- Female equivalent of seminoma
- Highly radiosensitive

81. Krukenberg Tumor

- Metastatic mucinous tumor to bilateral ovaries
- Most commonly originates from Gastric Carcinoma
- Signet ring cells on histology
- Presents as bilateral solid ovarian masses
- Very poor prognosis

82. Yolk Sac Tumor (Ovary)

- Malignant endodermal sinus tumor in young girls
- Schiller-Duval bodies
- Markedly elevated AFP
- Aggressive, rapid growth
- Presents with abdominal pain/mass

83. Granulosa Cell Tumor

- Most common malignant sex cord-stromal tumor
- Call-Exner bodies (cells arranged in small follicles)
- Coffee bean nuclei
- Produces Estrogen (precocious puberty in kids, endometrial hyperplasia in adults)
- Can cause abnormal uterine bleeding

84. Struma Ovarii (Specialized Teratoma)

- Monodermal teratoma composed of mature thyroid tissue
- Can cause clinical hyperthyroidism
- Presents as an ovarian mass
- Usually benign
- Shows thyroid follicles on histology

85. Pleomorphic Adenoma

- Most common salivary gland tumor (Parotid)
- Biphasic: Chondromyxoid stroma and epithelial cells
- Painless, mobile mass at angle of jaw
- High rate of recurrence if incompletely excised
- Rarely undergoes malignant transformation

86. Warthin Tumor

- Benign cystic tumor of salivary gland
- Papillary cystadenoma lymphomatosum
- Abundant lymphocytes and germinal centers
- Almost exclusive to smokers
- Frequently bilateral (10%)

87. Ameloblastoma

- Locally aggressive odontogenic tumor
- Arises from enamel-forming cells
- Soap bubble appearance in mandible on X-ray
- Palisading basal cells on histology
- High recurrence rate

88. Retinoblastoma

- Malignant tumor of the retina in children
- Leukocoria (white pupillary reflex) instead of red reflex
- Rb tumor suppressor gene mutation (Chr 13)
- Flexner-Wintersteiner rosettes
- Increases risk of Osteosarcoma later in life

89. Rhabdomyosarcoma

- Most common malignant soft tissue tumor in children
- Desmin and Myogenin positive
- Cross-striations seen on histology
- Sarcoma botryoides variant: Grape-like mass protruding from vagina
- Highly aggressive

90. Liposarcoma

- Most common malignant soft tissue tumor in adults
- Lipoblasts with scalloped nuclear margins
- Deep soft tissue or retroperitoneum
- MDM2 amplification
- Slow growing but can be very large

91. Synovial Sarcoma

- t(X;18) translocation
- Biphasic histology (spindle cells and epithelial-like glandular cells)
- Occurs near the knee in young adults
- Despite the name, does not arise from synovium
- Aggressive with early metastasis to lungs

92. Carcinoid Heart Disease

- Result of systemic carcinoid syndrome (liver mets)
- Right-sided endocardial fibrosis
- Tricuspid regurgitation and Pulmonary stenosis
- Spares left heart (serotonin inactivated by lung MAO)
- Presents with right heart failure

93. Atrial Myxoma

- Most common primary cardiac tumor in adults
- Usually located in the left atrium
- Gelatinous material, scattered spindle cells
- Presents with syncope or a 'tumor plop' sound
- Can embolize and cause strokes

94. Cardiac Rhabdomyoma

- Most common primary cardiac tumor in children
- Strongly associated with Tuberous Sclerosis
- Spider cells on histology
- Usually located in the ventricles
- Often regresses spontaneously

95. Angiosarcoma

- Malignant vascular tumor
- Hepatic variant associated with Vinyl chloride and Arsenic
- Cutaneous variant: Sun-exposed areas of elderly
- Stewart-Treves syndrome: Post-mastectomy lymphedema leading to angiosarcoma
- Very aggressive, bleeds easily

96. Insulinoma

- Pancreatic neuroendocrine tumor
- Produces excess insulin
- Whipple triad (hypoglycemia, CNS symptoms, relief with glucose)
- Elevated C-peptide (distinguishes from exogenous insulin)
- Mostly benign

97. Glucagonoma

- Pancreatic neuroendocrine tumor
- Overproduction of glucagon
- Necrolytic Migratory Erythema (classic rash)
- Diabetes mellitus (mild)
- Weight loss and DVT risk

98. VIPoma

- Non-beta cell pancreatic tumor secreting VIP
- WDHA syndrome (Watery Diarrhea, Hypokalemia, Achlorhydria)
- Pancreatic cholera
- Severe dehydration
- Treat with Octreotide

99. Somatostatinoma

- Pancreatic delta-cell tumor
- Inhibits insulin, glucagon, gastrin, and CCK
- Diabetes mellitus
- Steatorrhea and Gallstones (decreased CCK)
- Achlorhydria

100. Hydatidiform Mole (Complete)

- 46, XX or 46, XY (All chromosomes are paternal)
- No fetal tissue present
- 'Snowstorm' appearance on ultrasound
- Markedly elevated hCG, large uterus for dates
- 2% risk of transformation to Choriocarcinoma